



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : فاطمة محمد فوزى ابوالحسن
المحافظة : المنوفية الحي / المدينة : اشمون
رقم البطاقة : 28504051700787 رقم التليفون : 01007077660
العنوان : مدينة نصر القاهرة
العمر (تاريخ الميلاد) : 05-04-1985 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 28-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي :

نتائج الأبحاث : (مرفق)

التشخيص النهائي :

خطة العلاج : أبحاث أورام تصوير مقطعي محوسب مع صبغة للصدر والبطن
والحوض [11129]
التعليق على التشخيص : -----
قسم العلاج :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : -----
الاسم : د/ندى حسن الوظيفة : -----
الاسم : د/نرمين رمضان الوظيفة : -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: فاطمة محمد فوزى ابوالحسن National ID: 28504051700787	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

A patient of uterine cancer for Contrast-enhanced CT scan of the chest, abdomen, and pelvis.
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



فاطمة
محمد فوزى ابوالحسن
ب ٩٨ مدخل ٤ الحى السادس
مدينة نصر اول - القاهرة

١٩٨٥/٠٤/١٠
٢٨٥٠٤٠٥١٧٠٠٧٨٧
LH5336999

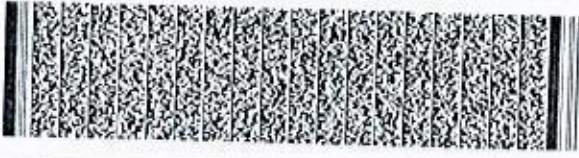
٢٠٢٥/٠٨ ٢٨٥٠٤٠٥١٧٠٠٧٨٧



ربة منزل
أنثى
مسلمة
متزوجة
اسلام حسن النبوي حسن
البطاقة سارية حتى ٢٠٣٢/٠٨/١١



١



Patient Name: Fatma Mohamed Fawzy Abu Al Hassan	Date: 01/10/2025
Exam: CT CAP+C (CT Chest Abd Pelvis)	Age: 40 Year(s)
Medical ID: 896/2025	Pat. ID: 121833

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS

REVEALED:

Status post TAH-BSO and infra colic omentectomy with no available pathological data or previous films.

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Post operative status showing clear operative bed with no residual/ recurrent masses.
- Average sized liver showing homogenous parenchymal attenuation, it shows a tiny right hepatic lobe segment VI hypodense focus measuring 4.5 x 2.9 mm. no dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few tiny scattered vertebral lytic foci and tiny D10 sclerotic focus.

pinion:

- Status post TAH-BSO showing clear operative bed.
- Tiny right hepatic lobe hypodense focus for triphasic CT/ dynamic MR assessment.
- Few tiny scattered vertebral lytic foci and tiny D10 sclerotic focus for bone scan.

Sherif Wagdy
Sherif Wagdy

Name: فاطمه محمد فوزى ابو الحسن			
MR. : 6836			
Detailed Medical Report			
		السيد	
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الخروج :		تاريخ وساعة الدخول :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is a 40-year-old with stage III B ovarian cancer, BRCA1-mutated, high-grade serous carcinoma. The patient underwent total abdominal hysterectomy with bilateral salpingo-oophorectomy on 6 April 2025, which revealed bilateral ovarian masses and positive omental deposits. Post-operative pathology confirmed high-grade serous carcinoma. The patient subsequently received adjuvant chemotherapy consisting of taxol and carboplatin for six cycles. After completion of chemotherapy, the patient was started on maintenance therapy with Lynparza in October 2025, continued through December 2025, and remained on stable disease as of January 2026. The patient has completed six cycles of the taxol-carboplatin regimen.			
تحريرا في ,			
التاريخ :		21-09-2026	
الوقت :			
dina makram		الطبيب المعالج :	
يعتمد ,			

Date : 01/10/2025	Patient Name: Fatma Mohamed Fawzy Abu Al Hassan
Age : 40 Year(s)	Exam : MAMMOGRAPHY BOTH
Pat. ID: 121833	Medical ID:

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: history of ovarian cancer , for routine screening .

Findings:

Right breast.

- Extremely dense breast lower the sensitivity of the mammography. (ACR D)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Extremely dense breast lower the sensitivity of the mammography. (ACR D)
- No speculations, masses asymmetry , distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated heterogeneous breast with mottled appearance with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

- **Right breast (BIRADS 2): breast adenosis.**
- **Left breast (BIRADS 2): breast adenosis.**

Recommendation:-

- **Right breast: for annual screening.**
- **Left breast: for annual screening.**

Best Regards,
DR. RANIA AHMED, MD
Signature Dr. Amal Hegazy
01/10/2025 12:21 PM



خطابات أورام

Nuclear Medicine Unit

Patient Name: Fatma Mohamed Fawzi

Date: 18/10/2025

^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Referring physician: Prof.

Technique: Anterior and posterior whole-body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

- No definite active osseous metastatic bony lesions
- Abnormal heterogenous radiotracer uptake is seen at D/L spine , more advanced at mid dorsal , lumbar vertebrae , likely of benign degenerative changes .
- Bilateral shoulders , both knees , joints are seen with osteoarthritic changes , .
- Both kidneys are normally seen.

Conclusion:

- No evidence of active osseous bony lesions could be seen all over the surveyed body ,

Nuclear Medicine Unit

DR. Sherien Mahdy, MD

DR/ Mohamed Emam

Patient Name : الأستاذة / فاطمة محمد فوزي ابو الحسن	Request No. : 13004507
Patient No. : 28504051700787	Request Date : 06/04/2025
Path No. : 1191/25	Verification Date : 30/04/2025
Séx : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 40 Y	Department : Out Patient
Referreing Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

- The cervix showed chronic non specific inflammation & free
- The endometrium was formed of uneven tubular and rounded glands surrounded by spindled stroma and free
- 2- Right lymph nodes: Free reative nodes (0/3)
- 3- Left lymph node: Free reactive nodes (0/8)
- 4- Cytological examination revealed inflammatory cells and reactive mesothelial cells in proteinous background

* IPT:

- WT1: Positive
- P53: Mutant (Strong diffuse nuclear staining)
- ER and PR: Positive

Diagnosis:

Total abdominal hystrectomy and bilateral salpingo-oophorectomy, Infraolic omentetomy and Bilateral lymph node biopsy:

1- Bilateral ovarian high grade adenocarcinoma (serous type)

- The tumor infiltrating the omentum and serosal nodules in both uterine corpus and bladder.

- Chronic cervicitis

- Disoreded proliferative endometrium

2- Right lymph nodes: Free (0/3)

3- Left lymph nodes: Free (0/8)

4- Cytology: Mild inflammatory smears with reactive mesothelial cells in a protinaous background

* FIGO stage: III B

Signature

Dr. Rana Fathy

Dr. Marwa Mahmoud

Verified by :

Dr.Toka Medhat

Printing Date : 05/05/2025 11:37:49

Page 2 of 3

Printing By : .marwa mohamed youss



غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mistr El Qadima - Cairo, Egypt

Patient Name : الأستاذة / فاطمة محمد فوزي ابو الحسن

Patient No. : 28504051700787

Path No. : 1191/25

Sex : Female

Age : 40 Y

Referring Doc :

Request No. : 13004507

Request Date : 06/04/2025

Verification Date : 30/04/2025

Financial Category : نفقة الدولة - اورام دار السلام

Department : Out Patient

Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical data: A case of ovarian cancer

Nature of specimen: Total abdominal hysterectomy, Bilateral salpingo-oophorectomy, infracolic omentectomy and bilateral lymph node biopsy.

Gross Examination:

1- Total abdominal hysterectomy and Bilateral salpingo-oophorectomy measured as followed:

- Uterine corpus measured 6x6x2 cm, cervix 3.5x2.5 cm.

right ovary measured 8x6x3 cm, right tube measured 7 cm long, left ovary measured 12x11x2 cm, left tube measured 6 cm long, both ovaries were distended, totally replaced by soft to rubbery tan mass with capsular disruption grossly, cut section was necrotic hemorrhagic.

- On opening uterine cavity measured 5x3.5cm, endometrium measured 0.2 cm thick, myometrium measured 1.8 cm thick showing trabeculations.

- Also anterior uterine serosal surface showed nodule measured 0.5x0.5 cm.

2- Right lymph nodes: Fibrofatty tissue piece measured 4x4x1 cm, cut section revealed 3 nodules up to 0.5x0.5 cm.

3- Left lymph nodes: Fibrofatty tissue piece measured 5x5x1 cm, cut section revealed 8 nodules up to 1.5x0.7 cm

4- Ascites: Two syringes 20 ml and 15 ml of hemorrhagic fluid.

Microscopic Examination:

1- Histopathological examination of both ovarian masses revealed malignant growth formed of sheets cribriform and papillae of markedly anaplastic epithelial cells having markedly pleomorphic bizarre nuclei with frequent mitosis and surrounded by wide necrosis.

- Both tubes and parametrium were free

- The omental nodules were positive for the same tumor

- The serosal nodules on surface of uterine corpus & bladder were positive for tumor

Verified by :

Dr.Toka Medhat

Printing Date : 05/05/2025 11:37:49

Page 1 of 3

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo, Egypt



Report

Patient Name: فاطمة محمد فوزي

Exam Date: 14-03-2025

MRI OF THE ABDOMEN AND PELVIS WITH IV CONTRAST:-

The study was performed on a 1.5 Tesla closed magnet system.

TECHNIQUE:-

Variable pulse sequences were taken.

FINDINGS:-

- Bilateral (large on the left) adnexal complex lesions measuring 11 x 9 cm on the left & 7 x 5 cm on the right side presenting mainly soft tissue component with multiloculated cystic component & presenting 3 x 3 cm hemorrhagic cyst on the right side (hyperintense on T1 & T2).
- The soft tissue component appears hypointense on T1 & T2 & presents inhomogenous enhancement after gadolinium administration.
- No peri- or para- aortic adenopathy.
- Normal signal void flow of the major abdominal vessels.
- Normal MRI appearance of the urinary bladder and uterus.
- Moderate ascites is noted.
- The liver is of normal MRI signal pattern, average size with no focal lesions.
- No intra- or extra-hepatic biliary dilatation.
- Normal MRI signal of the spleen with no focal lesions.
- The pancreas is of normal MRI appearance.
- Normal MRI appearance of both kidneys, that show preserved parenchyma with no evidence of focal lesions, no stones or back pressure changes elicited.

OPINION:-

- Bilateral (large on the left) adnexal complex lesions with moderate ascites, picture suspicious of ovarian neoplastic process for clinical correlation & further work up.

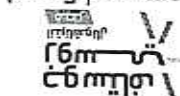
MUCH OBLIGED

PROF, DR. MOHAMED AMIN

DR. HOSSAM FARAHAT, M.D

CHECKED BY DR:

Typed by: R.M



Patient's Name:

Fatima Mohammed Fawzi

Date:

2026-07-16

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Post TAH BSO status with no operative bed masses or collections
- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ No hepatic or pulmonary metastatic lesions.

Dr.Asser Ahmed

Dr.Asser Ahmed

MDT FORM

Patient Name		عبدالله محمد فوزي		Age	40
Sex		Special habits		Comorbidity	hypothyroidism
Family History		Telephone		Marital Status	
Surgical History		Medical ID	28504051700787	Reason for MDT	896/2025
Menopausal Status		Date	28/10/2025		

Affected breast:		Medical Report	
Pathology:		Cancer ovary stage 3 B Undergone TAH, BSO on 6/4/2025 Bilateral ovarian mass +ve omental nodule high grade ovarian serous adenocarcinoma	
CT free SM free BRCA 1 mutant Recommendations: Received adjuvant CTH taxol , carboplatin for 18 wks ***25/9/2025 For MDT And risk reduction surgery		MDT DECISION For: counselling the patient about risk reduction bilateral mastectomy +/- reconstruction	

Name	أحمد محمد فوزي
Signature	25/10/2025 Dr. Most

Patient Name: Fatma Mohamed Fawzy Abu Al Hassan Exam : TRIPHASIC ABDOMEN & PELVIS Medical ID: 896/2025	Date : 12/10/2025 Age : 40 Year(s) Pat. ID: 121833
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MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, WITH TRIPHASIC STUDY OF THE ABDOMEN AND PELVIS REVEALED:

Status post TAH-BSO and infra colic omentectomy with no available pathological data or previous films, compared to last study dated 1/10/2025

I- CHEST:

Stationary course regarding:

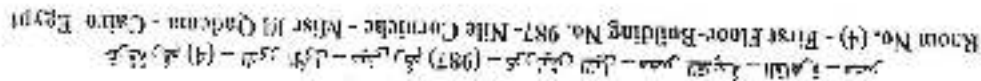
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- No residual/ recurrent masses.
- Average sized liver showing homogeneous parenchymal attenuation, no enhancing focal lesions .
- No intrahepatic biliary radicals dilatation.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the rest of pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few tiny scattered vertebral lytic foci and tiny D10 sclerotic focus.

Opinion:

- ❖ Status post TAH-BSO showing clear operative bed.
- ❖ Mild hepatomegaly , no enhancing focal lesions .
- ❖ Few tiny scattered vertebral lytic foci and tiny D10 sclerotic focus for bone scan



Printing By : [marwa mohamed youssef](#)

Microscopic Examination:

- 1- Histopathological examination of both ovarian masses revealed malignant growth formed of sheets cubiform and papillae of markedly anaplastic epithelial cells having markedly pleomorphic bizarre nuclei with frequent mitosis and surrounded by wide necrosis.
- Both tubes and parametrium were free
- The omental nodules were positive for the same tumor
- The serosal nodules on surface of uterine corpus & bladder were positive for tumor

Gross Examination:

- 1- Total abdominal hysterectomy and Bilateral salpingo-oophorectomy measured as followed:
 - Uterine corpus measured 6x6x2 cm, cervix 3.5x2.5 cm.
 - right ovary measured 8x5x3 cm, right tube measured 7 cm long, left ovary measured 12x11x2 cm, left tube measured 6 cm long, both ovaries were distorted, totally replaced by soft to rubbery tan mass with capsular disruption grossly, cut section was necrotic hemorrhagic.
- On opening uterine cavity measured 5x3.5cm, endometrium measured 0.2 cm thick, myometrium measured 1.8 cm thick showing trabeculations.
- Also anterior uterine serosal surface showed nodule measured 0.5x0.5 cm.
- 2- Right lymph nodes: Fibrotatty tissue piece measured 4x4x1 cm, cut section revealed 3 nodes up to 0.5x0.5 cm.
- 3- Left lymph nodes: Fibrotatty tissue piece measured 5x5x1 cm, cut section revealed 8 nodes up to 1.5x0.7 cm
- 4- Ascites: Two syringes 20 ml and 15 ml of hemorrhagic fluid.

Clinical data: A case of ovarian cancer
Nature of specimen: Total abdominal hysterectomy, Bilateral salpingo-oophorectomy, infracolic omentectomy and bilateral lymph node biopsy.

Pathology Report

Patient Name : مريم بنت محمد Patient No. : 28504051700787 Path. No. : 1191/25 Sex : Female Age : 40 Y	Request No. : 13004507 Request Date : 06/04/2025 Verification Date : 30/04/2025 Financial Category : مريض - Patient Department : Out Patient Registered by : Mrs. Mayada Hegazy
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Presidential Institute for Women's Health - Pathology Lab
 10000 Wilshire Blvd - Suite 2000 - Beverly Hills, CA 90210

Pathology Report
[[DOCTYPE:PATHOLOGY REPORT]]



Room No. (4) - First Floor-Building No. 987-Nile Corniche - Atter El Qadima - Cairo, Egypt
مركز الطب - طابق الأول - مبنى رقم 987 - نيل كورنيش - أتر القديما - القاهرة، مصر

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Verified by :
Dr. Toka Medhat

Dr. Marwa Mahmoud

Signature

Dr. Rana Fathy

* FIGO stage III B

Diagnosis:
Total abdominal hysterectomy and bilateral salpingo-oophorectomy, intrafolic omentectomy and Bilateral lymph node biopsy;
1- Bilateral ovarian high grade adenocarcinoma (serous type)
- The tumor infiltrating the omentum and serosal nodules in both uterine corpus and bladder.
- Chronic cervicitis
- Dispersed proliferative endometrium
2- Right lymph nodes: Free (0/3)
3- Left lymph nodes: Free (0/8)
4- Cytology: Mild inflammatory smears with reactive mesothelial cells in a proteinaceous background

- ER and PR: Positive
P53: Mutant (Strong diffuse nuclear staining)
- WT1: Positive
- IPT:
proteinaceous background
4- Cytological examination revealed inflammatory cells and reactive mesothelial cells in
3- Left lymph node: Free reactive nodes (0/8)
2- Right lymph nodes: Free reactive nodes (0/3)
spindled stroma and free
- The endometrium was formed of uneven tubular and rounded glands surrounded by
- The cervix showed chronic non specific inflammation & free

Pathology Report

Referring Doc :	Registered by :	Request No. :	Request Date :	Verification Date :	Financial Category :	Sex :	Age :
	Mrs. Mayada Hanyazy	13004507	06/04/2025	30/04/2025	Female	40 Y	
Path No. :	1191/25	28504051700787	06/04/2025	30/04/2025	Female	40 Y	
atient No. :	28504051700787						
atient Name :	د. محمد فوزي / د. محمد فوزي						